

South Asia. There was negligible regional collaboration and information exchange, and no stable platform for joint action. An initial convening of the heads of states of countries in the South Asian Association for Regional Cooperation (SAARC) in March 2020 led to the creation of an extremely modest emergency covid-19 fund of around \$11.5m, mainly based on voluntary contributions.¹⁹ However, little of this money has been disbursed, and tangible collaboration in surveillance, information exchange, or early warning of emerging risks from variants of concern has been negligible. No large collaborative international projects have been launched by science bodies and research centres in South Asia.

Although there were remarkable exemplars of resilience and stable national public health programmes such as the BRAC outreach workers²⁰—community health workers in Pakistan and India²¹—there were few common learning platforms specific to South Asia (despite the plethora of webinars with participants from the region) and no functional networks of public health institutions. Glaringly, unlike in other regions, there have been no data sharing exercises across South Asia related to covid-19 and vaccination coverage.

The capacity of the region to mount an emergency response to covid-19 partly depended on national and regional development of rapid diagnostic capacities; production of much needed personal protection equipment, oximeters, and ventilators; and a reliable oxygen production and supply chain across the region. Although India rapidly developed its own testing capacities, several South Asian countries spent a fortune importing expensive reagents and testing kits that could have been procured regionally at a fraction of the cost.²² The varying capacities within the region in vaccines and biological medicines could not have been starker; political differences and conflict pipped public health measures and international cooperation. India rapidly positioned itself to become the mainstay of the global supply chain for Covax vaccine distribution²³ and launched regional vaccine donations to South Asian neighbours, explicitly excluding Pakistan.²⁴ However, this vaccine diplomacy was also short lived as a rise in cases related to the delta variant led India to stop exports completely.

Collaboration, common learning, and shared experience could have improved other aspects of the pandemic response. For example, one of the serious harms of covid-19 mitigation strategies such as lockdowns has been their economic impact on people on low incomes, with social safety nets insufficient to meet the challenges. The first lockdown in India was associated with massive social upheaval and population displacement from cities to rural areas,²⁵ and although this was followed by a massive ramp up and expanded reach of food and other safety nets, the effects on food security remain.²⁶ Similar effects were noted in Bangladesh.²⁷ In contrast, Pakistan rapidly executed a programme of emergency cash transfers,²⁸ blunting much of the early economic shock and permitting the government to institute mitigation measures at scale. An early and free exchange of ideas and options across the region could have led to a much more coherent policy response and mutual learnings to the benefit of those on low incomes.

Potential role of health diplomacy

The ability of health related efforts to drive long term peace and stability has been questioned.²⁹ However, such efforts have

previously paused hostilities in Africa and Latin America to allow for humanitarian assistance and mass immunisations (such as for smallpox and polio).³⁰ Public health imperatives have previously surmounted conflict in South Asia, including the massive outpouring of support and sympathy from the region during the Pakistan earthquake in 2005³¹ and recent responses from civil society in Pakistan in the wake of the covid-19 disaster in India.³²

For their part, health professionals often already work in close collaboration with local, regional, and global civil society institutions and networks to promote peace, health, and wellbeing. Voices from the International Physicians for the Prevention of Nuclear War and International Campaign to Abolish Nuclear Weapons³³ and others³⁴ have been advocating for peace and de-escalation in the region for years. Public health and development professionals in the region must come together to strengthen learning and support regional peace and security as critical ingredients for South Asia's human, economic, and social development. Numerous examples exist of health experts from different countries mobilising to provide humanitarian assistance to neighbouring countries in times of crises, and they need to do the same in responding to covid-19. Improved technology now permits much easier intraregional interaction digitally.

Tackling poverty and inequity

Although the main goals of health diplomacy are to promote health by enhanced interaction and collaboration between states and civil society organisations and to improve health security and population health, an explicit focus on reducing poverty and inequities gives the exercise much greater legitimacy and appeal. Furthermore, progress towards peace and security cannot be made without tackling root causes of conflict such as poverty, corruption, and inequalities; ensuring application of the rule of law; respect for human rights; small arms control; and improved governance. Reducing inequities and social marginalisation could also help mitigate important root causes of obscurantism and militancy.

If there is one lesson from covid-19 across the world, it is that systemic inequities and disparities led to much of the avoidable excess mortality. As we advocate for restoration of peace, confidence building, and strengthening public health services in South Asia, a clear focus must be on reducing health disparities and reaching the unreached. This requires investment in human resources and infrastructure for primary healthcare and education, both of which have been seriously hampered by the current pandemic. Redirecting public policy and funds towards unmet social needs and a renewed battle against poverty, hunger, disease, and illiteracy would surely be the most important triumph for a region that boasts 25% of the world's population and one of the largest clustering of economically disadvantaged people, including in urban areas.

Calls for strengthening of primary care and human development investments in South Asia are not new and were the centrepiece of the landmark Bhore committee recommendations as far back as 1946.³⁵ The unprecedented crisis created by covid-19 offers an opportunity to make gains on the inequities it laid bare. However, building back fairer after covid-19 will need a substantial reduction in military build-up and expenditure and a corresponding increase in expenditures on health and education across the region, which are very low presently (table 1).^{36–40}